

Ear infection

- Mastoiditis
- Mastoiditis is a bacterial infection of the mastoid bone behind the ear. Without treatment it can lead to meningitis and brain abscess.

- a tender swelling behind the ear which pushes the ear forward

- Diagnosis
- Key diagnostic features are:
 - High fever
 - Tender swelling behind the ear

- Treatment (WHO)
- Give IM or IV cloxacillin 50mg/kg every 6 h or ceftriaxone 50 mg
- /kg for total of ten days
- If there is no response to treatment within 48 hours or the child's
- condition deteriorates, consult surgical specialist to consider
- incision and drainage of mastoid abscesses or mastoidectomy.
- If there are signs of meningitis, give antibiotic treatment as in
- bacterial meningitis
- If brain abscess suspected start antibiotic and consult surgical
- specialist immediately

2 Acute Otitis Media

Diagnosis

Opaque, or perforated tympanic membrane
with discharge

- Give oral antibiotics in one of the following regimens:
- First choice: Oral amoxicillin at 40 mg/kg twice a day for at least 5
- days
- If sensitivity for co-trimoxazole is known, give co-trimoxazole
- daily for five days

- Ear discharge culture is important

Chronic Otitis Media

- If pus has been draining from the ear for 2 weeks or longer, the child has a
- chronic ear infection.

- This is based on a history of pus draining from the ear for more than 2 weeks. On examination, confirm chronic otitis media) by otoscopy

Treatment

- Keep the ear dry by wicking
- Instill topical antibiotic (with or without steroids) once daily for 2 weeks.

- Drops containing quinolones (norfloxacin, ciprofloxacin) are more effective than other antibiotic drops and can be applied for 14 days

